SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Trastuzumab IV · every 3 weeks

HER2-positive early or metastatic breast cancer · NCCP 00200 · version 8 · 21-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
In early breast cancer, trastuzumab helps lower the chance of recurrence. In metastatic disease, the aim is ongoing cancer control. Your team should name which pathway applies before discussing duration.

Schedule
IV trastuzumab every 21 days. Early breast cancer: complete one year total, allowing for earlier trastuzumab. Metastatic disease: continue while beneficial and tolerated; no automatic one-year stop.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Duration and treatment setting discussed
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Common, serious and lasting agent-specific risks discussed below
- ☐ Correct HER2 medicine and formulation confirmed
- ☐ Heart and other required monitoring explained
- ☐ Pregnancy / contraception / breastfeeding discussed where relevant
- ☐ Companion cancer medicines reviewed using their own information

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Infusion or allergic reaction
- ☐ Diarrhoea and fatigue
- ☐ Rare serious lung effects

Discuss expected benefit, alternatives, route, duration, cardiac monitoring, pregnancy / contraception, urgent contacts and individual treatment risks. Add any companion drug using its own regimen information.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00200 v8.

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